

Physician Clinical Narrative

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

09/22/2026 12:35:00

Patient: HARRIS, OLA M MRN#: A134531 Acct#: 561536 Sex: Female DOB: 11/05/1969 Age: 56y

Disposition: Transfer to Memorial Health Savannah Disposition Decision Time: 15:32 09/22/2026

Measurements

Wt: 90.7 kg

Initial Vital Sign

Measured Time	BP	MAP	HR	RR	O ₂ Sat	ETCO ₂	Temp	Pain	GCS	RTS
12:45 09/22/2026	140/80	100	130	12			93.2 F	0	10	

Time Seen: (1245).

Arrived- By ambulance. Historian- patient.

HISTORY OF PRESENT ILLNESS

Chief Complaint: DECREASED MENTAL STATUS. PATIENT WITH HISTORY OF DIABETES AND CHRONIC RENAL FAILURE, PERITONEAL DIALYSIS, FOUND UNRESPONSIVE BY SON. EMS FOUND ACCU CHECK 24 GAVE IV AMP D50, ACCUCHECK 25 UPON ARRIVAL. (HAS SPONTANEOUS RESPIRATIONS). The patient was found unresponsive. Is still present. (PATIENT TRANSFERRED TO PEADMONT HOSP OF MACON ON 9/14/2026 FOR EVALUATION OF ALTERED MENTAL STATUS WITH A HYPERDENSE INTRAPARENCHYMAL FOCUS INVOLVING THE LEFT OCCIPITAL LOBE MEASURING 2.8CM X 1.2CM , SON STATES PATIENT LEFT AMA PRIOR TO IMAGING.). (SON STATES PATIENT FELL LAST NIGHT. PATIENT DENIES USING INSULIN TODAY.). The patient was found unresponsive. No history of chronic dementia. No change in diabetic routine, alcohol recently, recent drug use or medication given prior to arrival. Dextro stick was not low prior to arrival. The patient has had weakness.

Similar symptoms previously. None.

Recent medical care: Not recently seen/assessed.

REVIEW OF SYSTEMS

GU: No difficulty with urination. **GI:** No abdominal pain, nausea, diarrhea or black stools. **THROAT:** No sore throat. **MUSCULOSKELETAL:** No joint pain. **RESPIRATORY:** No difficulty breathing, cough or sputum production. **CVS:** No chest pain. **NEUROLOGICAL:** No headache, head injury or dizziness. **CONSTITUTIONAL:** No fever. **SKIN:** No skin rash. **EYES:** No blurred vision. All other systems reviewed and are negative.

Pregnancy Status: Not pregnant.

PAST HISTORY

- COPD - Chronic Obstructive Pulmonary Disease
- Diabetes Mellitus
- Gastroparesis
- HIGH CHOLESTEROL
- Hypertension
- Peritoneal dialysis (procedure)
- Renal Failure

Surgeries:

surgical history is unknown

Medications:

albuterol sulfate HFA 90 mcg/actuation aerosol inhaler: as needed.

Breztri Aerosphere inhalation

Brilinta 90 mg tablet: 90 mg twice a day .

calcitriol 0.5 mcg capsule: 0.5 mcg once a day .

citalopram 20 mg tablet: 20 mg once a day .

Fish Oil 1,000 mg (120 mg-180 mg) capsule: 1 cap once a day .

HumaLOG KwikPen Insulin subcutaneous: (SLIDING SCALE)

Lantus Solostar U-100 Insulin 100 unit/mL (3 mL) subcutaneous pen: 75 unit twice a day for type 2 diabetes mellitus.

Lipitor 40 mg tablet: 40 mg at bedtime .

megestrol 40 mg tablet: 40 mg once a day .

metoprolol succinate ER 50 mg tablet,extended release 24 hr: 50 mg at bedtime .

NIFEdipine ER 30 mg tablet,extended release: 30 mg once a day .

Norvasc 5 mg tablet: 5 mg once a day .

omeprazole 20 mg tablet,delayed release: 40 mg once a day .

potassium chloride ER 20 mEq tablet,extended release: 20 mEq once a day .

pregabalin 150 mg capsule: 150 mg twice a day .

pregabalin 25 mg capsule: 25 mg once a day .

QUETiapine 300 mg tablet: 150 mg at bedtime .

Singulair 10 mg tablet: 10 mg at bedtime .

Xphozah 30 mg tablet: 30 mg twice a day .

Allergies:

no known drug allergies

SOCIAL HISTORY

Heavy tobacco smoker- less than 1 pack per day. No drug use. Resides in a house. The patient lives with a family member.

FAMILY HISTORY

Hypertension in first-degree relative (father).

ADDITIONAL NOTES

The nursing notes have been reviewed.

PHYSICAL EXAM

Vital Signs: Have been reviewed.

Appearance: Alert. No acute distress.

Head: Head atraumatic. No tenderness or ecchymosis.

Eyes: Pupils equal, round and reactive to light. No dysconjugate gaze. No conjunctival findings or nystagmus. No pupillary exam or abnormal funduscopic findings.

ENT: Normal ENT inspection. Airway intact. Moist mucous membranes. Pharynx normal. No trouble handling secretions or pharyngeal erythema. The mucous membranes are not dry. Normal ear exam.

Neck: Normal inspection. No meningeal signs, carotid bruit or lymphadenopathy.

CVS: Normal heart rate. Heart sounds normal. Pulses normal.

Respiratory: No respiratory distress. No respiratory distress. Painless inspiration. Breath sounds normal. No decreased air movement, crackles or wheezes.

Abdomen: Soft. No obesity, abdominal tenderness or organomegaly. The bowel sounds are not abnormal.

Back: Normal inspection. No CVA tenderness.

Skin: No cyanosis. Skin warm. Normal skin color. Skin not cool on palpation. No skin rash.

Extremities: Extremities exhibit normal ROM. No lower extremity edema. No calf tenderness. No lower extremity edema.

Neuro: Alert. Oriented X 3. No alteration in mental status. Not disoriented. Alertness is not decreased. Verbal response is not abnormal. Response to pain is not abnormal. No aphasia. Mood/affect normal. Speech normal. No dysphasia. Cranial nerves normal (as tested). No cranial nerve deficit, facial weakness or decrease in corneal reflex. No cerebellar findings. No abnormal finger-nose test. No motor deficit. No weakness. No sensory deficit. No sensory deficit. Reflexes normal. Normal gait. No depression of the gag reflex. No pronator drift.

LABS, X-RAYS, AND EKG

Diagnostic Tests: Diagnostic tests have been ordered, with results reviewed and considered in the medical decision making process.

Laboratory Tests: PATIENT GIVEN GLUCAGON 1MG IM, AND EJ ACCESS, AMP D50, ACCUCHECK 57, IV D5NS AT 100ML/HR

1348 ACCUCHECK 131

PATIENT TRANSFERRED 9/14/2026 FOR ALTERED MENTAL STATUS, HEAD CT C/W A 2.8 X1.2 CM HEMORRHAGIC NON MASS LIKE LESION WITH MILD SURROUNDING VASOGENIC EDEMA, HEAD CT SCAN TODAY SHOWS NO CHANGE

PATIENT RECEIVED MORPHINE 2MG IV EVERY HOUR X 3 DOSES.

Laboratory Tests:

ABG ANALYSIS	Corrected	COLL: 09/22/2026 13:19:00 EDT MsgRcvd: 09/22/2026 13:23 EDT		
Lab Test	Result	Reference	Status	Received
ABG ANALYSIS			Final	09/22/2026 13:23:18 EDT
ARTERIAL BLOOD GAS				
O2 ROUTE:	NRB MASK		Final	09/22/2026 13:23:18 EDT
O2 LPM/%:	100%		Final	09/22/2026 13:23:18 EDT
BIPAP/CPAP	N/A		Final	09/22/2026 13:23:18 EDT
VENT SETTINGS	N/A		Final	09/22/2026 13:23:18 EDT
PUNCTURE SITE	L RADIAL		Final	09/22/2026 13:23:18 EDT
ALLEN TEST	POSITIVE		Final	09/22/2026 13:23:18 EDT
pH	7.43	7.35 - 7.45	Final	09/22/2026 13:23:18 EDT
pCO2	32.0 mm/HG Below low normal	35.0 - 45.0	Final	09/22/2026 13:23:18 EDT
pO2	78.0 mm/HG Below low normal	80.0 - 100	Final	09/22/2026 13:23:18 EDT
BASE EXCESS	-2.2	-2.4 - 2.3	Final	09/22/2026 13:23:18 EDT
TCO2	22.2 mmol/L Below low normal	25.7 - 29.2	Final	09/22/2026 13:23:18 EDT
HCO3	21.2 mEq/L Below low normal	22.0 - 28.0	Final	09/22/2026 13:23:18 EDT
O2 SAT	97.2 %	95.0 - 98.0	Final	09/22/2026 13:23:18 EDT
A-a GRADIENT	0.1 mm/HG Below low normal	10.0 - 20.0	Final	09/22/2026 13:23:18 EDT
CRITICAL RESULT	YES		Final	09/22/2026 13:23:18 EDT
{ HANDED TO PHYSICIAN DR OATES . { CALLED TO PHYSICIAN . { RBV BY MD/DELEGATE DR OATES . { DATE OF CONTACT 09/22/2026 . { TIME OF CONTACT 1323 .				
INIT/SIGN	CP RRT		Final	09/22/2026 13:23:18 EDT

BNP	Final	COLL: 09/22/2026 12:45:00 EDT MsgRcvd: 09/22/2026 13:57 EDT		
Lab Test	Result	Reference	Status	Received
BNP	211 pg/mL Above high normal	0 - 100	Final	09/22/2026 13:57:02 EDT
\BLD\B-TYPE NATRIURETIC PEPTIDE INTERPRETATION\BLDx\ The Beckman Coulter DXC 600i BNP test is used as an aid in the diagnosis and assessment of severity of congestive heart failure. The test is also used for the risk stratification of patients with acute coronary syndromes. NORMAL RANGES ABNORMAL RANGES <=100 pg/mL >100 pg/mL As of Feb. 10, 2014 analysis is being performed on the Beckman Coulter DXC 600i.				

CBC AUTO DIFF	Final	COLL: 09/22/2026 12:45:00 EDT MsgRcvd: 09/22/2026 13:25 EDT		
Lab Test	Result	Reference	Status	Received
CBC AUTO DIFF			Final	09/22/2026 13:25:49 EDT
COMPLETE BLOOD COUNT				
WBC	10.3 K/uL	4.0 - 10.8	Final	09/22/2026 13:25:49 EDT
RBC	4.40 M/uL	3.90 - 5.40	Final	09/22/2026 13:25:49 EDT
HEMOGLOBIN	12.8 g/dL	12.0 - 16.0	Final	09/22/2026 13:25:49 EDT
HEMATOCRIT	37.9 %	35.0 - 47.0	Final	09/22/2026 13:25:49 EDT
MCV	86 fL	80 - 100	Final	09/22/2026 13:25:49 EDT
MCH	29.1 pg	27.0 - 33.0	Final	09/22/2026 13:25:49 EDT
MCHC	34 g/dL	31 - 36	Final	09/22/2026 13:25:49 EDT
RDW	15.3 %	11.5 - 15.5	Final	09/22/2026 13:25:49 EDT
PLATELETS	438 K/uL Above high normal	139 - 358	Final	09/22/2026 13:25:49 EDT
MPV	8.5 fL	7.4 - 10.4	Final	09/22/2026 13:25:49 EDT
%NEUT	78 % Above high normal	40 - 77	Final	09/22/2026 13:25:49 EDT
%LYMPH	13 % Below low normal	14 - 48	Final	09/22/2026 13:25:49 EDT
%MONO	8 %	4 - 11	Final	09/22/2026 13:25:49 EDT
%EOS	0 % Below low normal	0 - 7	Final	09/22/2026 13:25:49 EDT

%BASO	1 %	0 - 1	Final	09/22/2026 13:25:49 EDT
#NEUT	8.0 K/uL Above high normal	1.9 - 7.3	Final	09/22/2026 13:25:49 EDT
#LYMPH	1.4 K/uL	0.9 - 3.8	Final	09/22/2026 13:25:49 EDT
#MONO	0.9 K/uL	0.2 - 0.9	Final	09/22/2026 13:25:49 EDT
#EOS	0.0 K/uL	0.0 - 0.5	Final	09/22/2026 13:25:49 EDT
#BASO	0.1 K/uL	0.0 - 0.1	Final	09/22/2026 13:25:49 EDT
MANUAL DIFF	NOT INDICATED		Final	09/22/2026 13:25:49 EDT
RBC MORPH	NOT INDICATED		Final	09/22/2026 13:25:49 EDT

{CD}

ANALYSIS PERFORMED ON BECKMAN-COULTER DXH900 AS OF AUGUST 15, 2022.

COMPREHENSIVE METABOLIC PANEL		Final	COLL: 09/22/2026 12:45:00 EDT MsgRcvd: 09/22/2026 13:34 EDT	
Lab Test	Result	Reference	Status	Received
COMPREHENSIVE METABOLIC PANEL			Final	09/22/2026 13:34:19 EDT
COMPREHENSIVE METABOLIC PANEL				
SODIUM	138 mEq/L	135 - 145	Final	09/22/2026 13:34:19 EDT
POTASSIUM	5.4 mEq/L Above high normal	3.5 - 5.0	Final	09/22/2026 13:34:19 EDT
CHLORIDE	99 mEq/L	98 - 112	Final	09/22/2026 13:34:19 EDT
CO2	27 mEq/L	21 - 31	Final	09/22/2026 13:34:19 EDT
GLUCOSE	50 mg/dL Below low normal	70 - 110	Final	09/22/2026 13:34:19 EDT
BUN	33 mg/dL Above high normal	7 - 21	Final	09/22/2026 13:34:19 EDT
CREATININE	11.62 mg/dL Above high normal	0.50 - 1.20	Final	09/22/2026 13:34:19 EDT
BUN/CREAT	2.84 Below low normal	6.00 - 25.00	Final	09/22/2026 13:34:19 EDT
CALCIUM	10.0 mg/dL	8.5 - 10.5	Final	09/22/2026 13:34:19 EDT
SGPT/ALT	11 IU/L	7 - 52	Final	09/22/2026 13:34:19 EDT

TOTAL BILI	0.3 mg/dL	0.0 - 1.3	Final	09/22/2026 13:34:19 EDT
TOTAL PROTEIN	6.9 g/dL	6.5 - 8.2	Final	09/22/2026 13:34:19 EDT
ALBUMIN	2.3 g/dL Below low normal	3.2 - 5.5	Final	09/22/2026 13:34:19 EDT
A/G RATIO	0.50 Below low normal	0.80 - 2.00	Final	09/22/2026 13:34:19 EDT
SGOT/AST	30 IU/L	13 - 39	Final	09/22/2026 13:34:19 EDT
ALKALINE PHOS	104 IU/L	42 - 121	Final	09/22/2026 13:34:19 EDT
AGE	56 yrs		Final	09/22/2026 13:34:19 EDT
NON-AA GFR	4 mL/min		Final	09/22/2026 13:34:19 EDT
AFR AMER GFR	4 mL/min		Final	09/22/2026 13:34:19 EDT

REFERENCE RANGE TABLE FOR POPULATION MEAN GFRs FROM NKDEP

AGE (years) AVERAGE GFR

20-29 116 ml/min/1.73m

30-39 107 ml/min/1.73m

40-49 99 ml/min/1.73m

50-59 93 ml/min/1.73m

60-69 85 ml/min/1.73m

70+ 75 ml/min/1.73m

Chronic kidney disease is defined as either kidney damage or GFR less than 60 for at least three months. eGFR should be correlated with patient's clinical status. eGFR should not be used to calculate drug dosage.

TROPONIN HS HIGH SENSITIVITY

Final

COLL: 09/22/2026 12:45:00 EDT MsgRcvd: 09/22/2026 13:49 EDT

Lab Test	Result	Reference	Status	Received
TROPONIN HS	46.9 pg/mL Above high normal	0.0 - 15.0	Final	09/22/2026 13:49:12 EDT

Potential for acute myocardial infarction when correlated with clinical suspicion for acute coronary syndrome.

PROTIME

Final

COLL: 09/22/2026 12:45:00 EDT MsgRcvd: 09/22/2026 15:25 EDT

Lab Test	Result	Reference	Status	Received
PROTIME	13.0 Secs Above high normal	10.7 - 12.6	Final	09/22/2026 15:25:21 EDT
INR	1.1	0.0 - 4.0	Final	09/22/2026 15:25:21 EDT

\BLDo\INR INTERPRETATION\BLDx\

Therapeutic range for Coumadin and related oral anticoagulants.

-International Normalized Ratio (INR): 2.0 - 3.0 for Venous Thrombosis, Pulmonary Embolus, Tissue heart valves, Acute MI Atrial Fibrillation, Valvular heart disease and recurrent Systemic Embolism.

-International Normalized Ratio (INR): 2.5 - 3.5 for Mechanical Prosthetic

valve.
As of 11/06/2023 coagulation studies at Bleckley Memorial Hospital are performed on the ACL TOPS 350.

Diagnostic Study Tests:

CT ABD PELVIS W/O CONTRAST

Final

MsgRcvd: 09/22/2026 14:41 EDT

BLECKLEY MEMORIAL HOSPITAL
145 E. PEACOCK STREET
COCHRAN, GA. 31014
-----NAME----- NUMBER SEX AGE ADMIT DISC. XRAY# F/C TYPE
HARRIS OLA M 561536 F 56 9/22/26 31633 MB E/R
DATE OF BIRTH: 11/05/1969 M/R# A134531 PH#: 478-230-1539 RM Wait3
LOCATION: INVALID TRANSCRIBED: 09/22/26 14:38
CT ABD PELVIS W/O CONTRAST 74176 COMPLETED:09/22/26 13:51 NOV 10116
ReasonForStudy: Abdominal Pain
PHYSICIAN: OATES MICH

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RADIOLOGY REPORT

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EXAMINATION:
CT ABD PELVIS W/O CONTRAST

HISTORY:
ABD PAIN; .

COMPARISON:
CT abdomen and pelvis 09/14/2026

TECHNIQUE:
Unenhanced axial images were obtained through the abdomen and pelvis using renal stone protocol. Reformatted images were obtained as well. Lack of oral and IV contrast limits diagnostic sensitivity
The above CT scan was done with automated exposure control and the mA and kV was adjusted to obtain quality images according to patient size.

FINDINGS:
Lung bases: No acute infiltrates. Patchy atelectasis in the right middle lobe and lower lobes. Respiratory motion.
Liver: No acute finding or focal lesion.
GB/Biliary: Contracted or surgically absent. No ductal dilatation
Spleen: Normal size and density
Pancreas: No acute findings. No pseudocyst or dilated duct
Adrenal Glands: No mass
Kidneys: Atrophic kidneys. No obstructing stone, hydronephrosis or solid lesion. Vascular calcification along the renal hila
Abdominal aorta: Tapers normally. Extensive atherosclerotic disease. Stent in the splenic artery
Retroperitoneum: No pathologically enlarged lymph nodes
Bowel: Peritoneal dialysis catheter. Tip in the midline pelvis. No displaced bowel or abnormal fluid collection. Trace of pneumoperitoneum is likely related to peritoneal dialysis. Suboptimal distention of the colon. No grossly thickened or dilated loops of bowel, free fluid, free air, pneumatosis or abscess. Moderate stool. No CT evidence for appendicitis, diverticulitis or obstruction
Bladder/GU: Ureters unremarkable. There is partial distention of the bladder. No pelvic or adnexal mass.
Osseous: Bony structures have the appearance of renal osteodystrophy. Multilevel degenerative changes. Discogenic sclerosis with vacuum disc phenomenon and Modic changes at L5-S1. No acute findings or aggressive bony lesions.

IMPRESSION:
No acute intra-abdominal or intrapelvic process. No change.
Peritoneal dialysis catheter with trace of free air likely related to

peritoneal dialysis.
No CT evidence for appendicitis, diverticulitis or obstruction
THIS IS AN ELECTRONICALLY VERIFIED FINAL REPORT
9/22/2026 2:38 PM - Electronically signed by William Willoughby, MD
Reviewed and Electronically Signed By:
WILLIAM WILLOUGHBY MD
RADIOLOGIST
Date/Time: 09/22/26 14:38

CT HEAD WO CONTRAST

Final

MsgRcvd: 09/22/2026 14:14 EDT

BLECKLEY MEMORIAL HOSPITAL
145 E. PEACOCK STREET
COCHRAN, GA. 31014
-----NAME----- NUMBER SEX AGE ADMIT DISC. XRAY# F/C TYPE
HARRIS OLA M 561536 F 56 9/22/26 31633 MB E/R
DATE OF BIRTH: 11/05/1969 M/R# A134531 PH#: 478-230-1539 RM Wait3
LOCATION: INVALID TRANSCRIBED: 09/22/26 14:11
CT HEAD WO CONTRAST 70450 COMPLETED:09/22/26 13:45 NOV 10117
ReasonForStudy: Altered Sense of Awareness
PHYSICIAN: OATES MICH

RADIOLOGY REPORT

EXAM:

CT HEAD WO CONTRAST

HISTORY:

ALTERED SENSE OF AWARENESS;

COMPARISON:

Head CT examination dated September 14, 2026.

TECHNIQUE:

Multiple helical images of the brain from the vertex to the occiput were obtained. Coronal and sagittal reformats were performed.
Dose reduction techniques including Automated Exposure Control (AEC) and adjustment of mA and kV were utilized.

FINDINGS:

There is a hyperdense intraparenchymal focus involving the left occipital lobe, measuring approximately 28 x 12 mm in greatest dimensions. This high-density lesion or hemorrhage demonstrates a predominantly linear configuration and is non-mass-like in appearance. Mild surrounding vasogenic edema is present. A small amount of adjacent subarachnoid hemorrhage is present within the posterior left occipital sulci.
These findings were present on the prior CT examination of September 14, 2026 and are essentially unchanged in size and appearance.
There is no significant associated mass effect or midline shift. The basal cisterns remain patent. There is no evidence of a herniation syndrome. No acute hydrocephalus is identified.
The remainder of the examination demonstrates no additional acute intracranial process, allowing for mild motion artifact. Mild scattered areas of decreased attenuation within the periventricular and subcortical white matter are compatible with chronic microvascular ischemic disease.
No additional intracranial hemorrhage or extra-axial fluid collection is identified. No CT evidence of a large acute territorial infarction is demonstrated.
Differential Considerations:
The left occipital intraparenchymal hemorrhage is nonspecific in etiology. Differential considerations include hemorrhagic transformation of an underlying ischemic infarction, hypertensive hemorrhage, underlying vascular malformation (including AVM or cavernous malformation), cerebral venous thrombosis/venous infarction, cerebral amyloid angiopathy, coagulopathy or anticoagulation-related hemorrhage, and less likely an underlying hemorrhagic primary or metastatic neoplasm.

IMPRESSION:

1. LINEAR, subacute appearing, NON-MASS-LIKE LEFT OCCIPITAL INTRAPARENCHYMAL HEMORRHAGE versus potential occipital lobe arteriovenous malformation, MEASURING APPROXIMATELY 28 X 12 MM WITH MILD SURROUNDING VASOGENIC EDEMA AND A SMALL AMOUNT OF ADJACENT LEFT OCCIPITAL SUBARACHNOID HEMORRHAGE. THESE FINDINGS WERE PRESENT ON SEPTEMBER 14, 2026 AND ARE ESSENTIALLY UNCHANGED FROM THE PRIOR EXAMINATION.

2. NO SIGNIFICANT ASSOCIATED MASS EFFECT, MIDLINE SHIFT, ACUTE HYDROCEPHALUS, OR EVIDENCE OF A HERNIATION SYNDROME.

3. DIFFERENTIAL CONSIDERATIONS INCLUDE an evolving HEMORRHAGIC INFARCTION, UNDERLYING VASCULAR MALFORMATION, HYPERTENSIVE HEMORRHAGE, CEREBRAL VENOUS THROMBOSIS/VENOUS INFARCTION, CEREBRAL AMYLOID ANGIOPATHY, COAGULOPATHY, AND LESS LIKELY AN UNDERLYING HEMORRHAGIC NEOPLASM.

4. MILD CHRONIC MICROVASCULAR ISCHEMIC WHITE MATTER DISEASE.

5. NEUROSURGICAL AND NEUROLOGY CONSULTATION IS RECOMMENDED. FOLLOW-UP MRI OF THE BRAIN WITHOUT AND WITH IV CONTRAST AND MRA OF THE BRAIN ARE SUGGESTED TO FURTHER CHARACTERIZE THE high-density lesion/hemorrhage AND EXCLUDE AN UNDERLYING STRUCTURAL OR VASCULAR ETIOLOGY.

THIS IS AN ELECTRONICALLY VERIFIED FINAL REPORT

9/22/2026 2:11 PM - Electronically signed by Beau Cannington, MD

Reviewed and Electronically Signed By:

ROYCE CANNINGTON MD

RADIOLOGIST

Date/Time: 09/22/26 14:11

PROGRESS AND PROCEDURES

Critical care performed (30 minutes). Time includes: direct patient care, patient reassessment, interpretation of data (laboratory data and chest xrays), review of patient's medical records and documentation of patient care. Procedures included in critical care time: peripheral IV placement.

Differential Diagnosis:

I considered epilepsy, alcohol related etiology, overdose, abnormal serum glucose and encephalitis as a possible cause of seizure in this patient.

Other possible considerations: FOUND UNRESPONSIVE BY SON, HYPOGLYCEMIA, HEAD CT C/W EVOLVING INFARCT INTRAPARENCHYMAL HEMORRHAGE WITH SURROUNDING VASOGENIC EDEMA, NO CHANGE IN PREVIOUS HEAD CT 8/14/2026 WHEN PATIENT TRANSFERRE TO PIEDMONT HOSP MACON FOR EVALUATION OF ALTERED MENTAL STATES, SON STATES PATIENT LEFT AMA PRIOR TO IMAGING.

Above considerations are based on history and physical exam.

Differential diagnosis was discussed with patient.

MEDICAL DECISION MAKING: Pertinent clinical findings include the acute presentation. The findings also include the risk factors for diabetes.

(UNREPOSIVE , AWAKENED AFTER AMP D-50 INTRAVENOUS). A serious condition is a probable cause for the patient's findings. The differential diagnosis includes, but is not limited to, multiple sclerosis, intracerebral hemorrhage and (HYPOGLYCEMIA). Ordered tests include a CT, chest x-ray and EKG, complete blood count, chemistries and troponin.

(ACCUCHECK IMPROVED FROM 25 TO 131).

Disposition: Disposition Decision Time: 15:32 09/22/2026. Patient transferred to Memorial Health Savannah. Transferred in stable condition. Decision for transfer based upon need for higher level of care. Benefits, risks and alternatives to transfer explained to patient and son. Case discussed with accepting provider DR RUFFO AT 1530. Information relayed regarding patient's vital signs, labs and radiology. Summary of care provided to transfer facility DISCUSSED DR RUFFO. Condition: stable.

CLINICAL IMPRESSION

EVOLVING INFARCTION,
UNDERLYING AVM
HYPOGLYCEMIA.

(Electronically signed by Michael Oates, M.D. 09/22/26 19:54:28 EDT)

Generated by CorroHealth

Nurse Narrative

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

09/22/2026 12:35:00

Patient: HARRIS, OLA	MRN#:	Acct#:	Sex:	DOB:	Age:
M	A134531	561536	Female	11/05/1969	56y
Disposition: Transfer to Memorial Health Savannah	Disposition Decision Time: 15:32 09/22/2026			Departure Time: 19:58 09/22/2026	

TRIAGE

Triage time: 12:35 09/22/2026. -- 13:41 09/22/26 EDT Lusha Fowler, R.N.

Historian: (EMS). Unaccompanied and arrived from home.

Acuity: LEVEL 2.

Chief Complaint: UNRESPONSIVE.

12:35 09/22/26.

Unknown when patient was last known well (21:30 09/21/2026).

EMS Treatment PTA:

EMS report unavailable. D-50 IVP: 25 gm, X1, given by EMS. (IV x2. AMBUING PT.). -- 14:23 09/22/26 EDT Lusha Fowler, R.N.

12:45 09/22/26. BP: 140/80 MAP: 100. HR: 130. RR: 12. Temperature: 93.2 F (rectal). Pain level now 0/10. Glasgow Coma Scale: 10 - eyes open to pressure (2); best verbal response - words (3); best motor response - localizing (5). --

14:24 09/22/26 EDT Lusha Fowler, R.N.

13:10 09/22/26. O2 saturation: 100% on non-rebreather. -- 15:50 09/22/26 EDT Lusha Fowler, R.N.

Measurements:

12:35 09/22/26 Wt: 90.7 kg -- 14:24 09/22/26 EDT Lusha Fowler, R.N.

Medications:

Lantus Solostar U-100 Insulin 100 unit/mL (3 mL) subcutaneous pen: 75 unit twice a day for type 2 diabetes mellitus. -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

Norvasc 5 mg tablet: 5 mg once a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

Brilinta 90 mg tablet: 90 mg twice a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

calcitriol 0.5 mcg capsule: 0.5 mcg once a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

citalopram 20 mg tablet: 20 mg once a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

metoprolol succinate ER 50 mg tablet,extended release 24 hr: 50 mg at bedtime . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

omeprazole 20 mg tablet,delayed release: 40 mg once a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

QUetiapine 300 mg tablet: 150 mg at bedtime . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

Singulair 10 mg tablet: 10 mg at bedtime . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

NIFedipine ER 30 mg tablet,extended release: 30 mg once a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

pregabalin 150 mg capsule: 150 mg twice a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

pregabalin 25 mg capsule: 25 mg once a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.
megestrol 40 mg tablet: 40 mg once a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.
potassium chloride ER 20 mEq tablet,extended release: 20 mEq once a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.
Xphozah 30 mg tablet: 30 mg twice a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.
Lipitor 40 mg tablet: 40 mg at bedtime . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.
Fish Oil 1,000 mg (120 mg-180 mg) capsule: 1 cap once a day . -- 14:32 09/22/26 EDT Lusha Fowler, R.N.
Breztri Aerosphere inhalation -- 14:32 09/22/26 EDT Lusha Fowler, R.N.
albuterol sulfate HFA 90 mcg/actuation aerosol inhaler: as needed. -- 14:32 09/22/26 EDT Lusha Fowler, R.N.
HumaLOG KwikPen Insulin subcutaneous: (SLIDING SCALE) -- 14:32 09/22/26 EDT Lusha Fowler, R.N.

12:35 09/22/26.

Preferred Pharmacy: MC DANIELS; COCHRAN. -- 14:23 09/22/26 EDT Lusha Fowler, R.N.

Allergies:

no known drug allergies -- 15:19 09/22/26 EDT Lusha Fowler, R.N.

Problems:

COPD - Chronic Obstructive Pulmonary Disease -- 16:13 09/22/26 EDT Lusha Fowler, R.N.
Diabetes Mellitus -- 16:13 09/22/26 EDT Lusha Fowler, R.N.
Hypertension -- 16:13 09/22/26 EDT Lusha Fowler, R.N.
Renal Failure -- 16:13 09/22/26 EDT Lusha Fowler, R.N.
HIGH CHOLESTEROL -- 16:13 09/22/26 EDT Lusha Fowler, R.N.
Gastroparesis -- 16:13 09/22/26 EDT Lusha Fowler, R.N.
Peritoneal dialysis (procedure) -- 16:14 09/22/26 EDT Lusha Fowler, R.N.

Surgeries:

surgical history unknown -- 15:29 09/22/26 EDT Lusha Fowler, R.N.

History

12:35 09/22/26.

SOCIAL HX: Unknown if ever smoked. The patient has not traveled outside the U.S.

Infectious disease exposure: No infectious disease exposure.

ABUSE ASSESSMENT: The patient answered "yes" to the question(s) "Do you feel safe in your home?" and "no" to the question(s) "Are you afraid to go home?". No suspicion of abuse.

SELF HARM ASSESSMENT: Self harm assessment was performed. The patient answered "no" to the question(s) "Are you here because you tried to hurt yourself?".

NUTRITIONAL RISK ASSESSMENT: The nutritional risk assessment revealed no deficiencies.

FUNCTIONAL ASSESSMENT: Functional assessment: no impairments noted.

LEARNING NEEDS ASSESSMENT: The learning needs assessment revealed no barriers.

SKIN INTEGRITY ASSESSMENT: Skin integrity risk assessment completed. No skin integrity risk identified. -- 14:23 09/22/26 EDT Lusha Fowler, R.N.
12:42 09/22/26.

PAST MEDICAL HX: LNMP: The patient is post-menopausal.
Denies current pregnancy. -- 15:49 09/22/26 EDT Lusha Fowler, R.N.

Interventions

12:35 09/22/26. Identification band on patient. -- 14:23 09/22/26 EDT Lusha Fowler, R.N.

PHYSICAL ASSESSMENT

12:35 09/22/26. To room via stretcher.

GENERAL / NEURO / PSYCH: (PT UNRESPONSIVE. EMS BAGGING PT).

RESPIRATORY: Moderate respiratory distress. (RESP M< 10 BPM PER EMS).

CVS: Cardiac rhythm: sinus tachycardia; (SINUS TACH WITH FREQUENT PVC's). Capillary refill less than 2 seconds.

GI / GU: Abdomen soft.

SKIN: Skin is warm and dry. Poor skin turgor. -- 13:41 09/22/26 EDT Lusha Fowler, R.N.

NURSING PROGRESS NOTES

late entry - 12:00 09/22/26. (PT ABLE TO ANSWER QUESTIONS. PT STATES HAS NOT HAD ANY INSULIN SINCE YESTERDSAY. PT CALLING FOR "BELLA" WHO IS PT'S DOG(PEWR SON)). -- 14:19 09/22/26 EDT Lusha Fowler, R.N.

12:35 09/22/26. Call light placed in reach. Side rails up x 2. Bed placed in lowest position. Brakes of bed on. -- 13:32 09/22/26 EDT Lusha Fowler, R.N.

12:42 09/22/26. Bedside glucose 24 mg/dL; performed by nurse via finger. Result shown to the ED physician. -- 13:33 09/22/26 EDT Lusha Fowler, R.N.

12:44 09/22/26. Glucagon IM 1 mg given. Given in the left anterior lateral thigh. Allergies verified and confirmed 5 rights. Information reviewed with patient and family including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 15:39 09/22/26 EDT Lusha Fowler, R.N.

12:45 09/22/26. Site #1 started in the left external jugular with an 18g needle with aseptic technique and good blood return; 1 attempt. Blood drawn: green, purple, blue and yellow tube(s). Saline lock flushed with 10 mL saline. (INT PLACED BY M.LAMB,LPN.) -- 13:35 09/22/26 EDT Lusha Fowler, R.N.

12:46 09/22/26. Dextrose 50%-Water IVP 25 g given via Site# 1. Allergies verified and confirmed 5 rights. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre-medication administration. IVP given by nurse. Information reviewed including reason for taking this medication and precautions. -- 15:41 09/22/26 EDT Lusha Fowler, R.N.

12:51 09/22/26. IV D5NS started in bag#1 1000 mL at 100 mL/hr via Site# 1. Allergies verified and confirmed 5 rights. Via IV pump. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre-medication administration. Information reviewed with patient and family including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 15:38 09/22/26 EDT Lusha Fowler, R.N.

12:53 09/22/26. ABG attempt unsuccessful by respiratory therapist from the right radial artery. Post-procedure pressure applied by respiratory therapist. -- 13:36 09/22/26 EDT Lusha Fowler, R.N.

13:10 09/22/26. ABG specimen obtained by respiratory therapist from the left radial artery. Post-procedure pressure applied by respiratory therapist. Post-procedure: pulse present, no bleeding and specimen labeled in presence of the patient and sent to lab. -- 13:36 09/22/26 EDT Lusha Fowler, R.N.

13:11 09/22/26. (BEAR HUGGER APPLIED DUE TO RECTAL TEMP 93.2). -- 13:37 09/22/26 EDT Lusha Fowler, R.N.

13:30 09/22/26. (DR.OATES STATES AS PT IS AWAKING SHE IS ABLE TO COME OFF EVERYTHING AND GO TO CT). -- 13:37 09/22/26 EDT Lusha Fowler, R.N.

13:30 09/22/26. Patient transported to CT by stretcher with radiology tech. -- 13:38 09/22/26 EDT Lusha Fowler, R.N.

13:31 09/22/26. (PT SON HERE-HE STATES PT CONNECTED HERSELF TO PERITONEAL DIALYSIS LAST NIGHT AND WAS STILL CONNECTED WHEN EMS ARRIVED TO HOUSE AND EMS DISCONNECTED DIALYSIS). -- 13:42 09/22/26 EDT Lusha Fowler, R.N.

13:44 09/22/26. Patient returned from CT by stretcher with radiology tech. -- 13:44 09/22/26 EDT Lusha Fowler, R.N.

13:50 09/22/26. Bedside glucose 131 mg/dL; performed by nurse via finger. Result shown to the ED physician. -- 14:12 09/22/26 EDT Lusha Fowler, R.N.

15:06 09/22/26. Tylenol PO 650 mg given. Allergies verified and confirmed 5 rights. Information reviewed with patient and family including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 15:12 09/22/26 EDT Lusha Fowler, R.N.

15:18 09/22/26. (WRITER SPOKE WITH ALANDRIA AT ONESTEP TRANSFER CENTER FOR POSSIBLE TRANSFER TO SAVANNAH MEMORIAL FOR NEUROSURGERY/TRAUMA EVALUTION AFTER HEAD CT FINDINGS AND PATIENT FAMILY STATES THE PATIENT DID HAVE A FALL LAST NIGHT AND POSSIBLY HIT HER HEAD.). -- 15:18 09/22/26 EDT Cheryl Hulette, L.P.N.

15:40 09/22/26. (1524- PT WAS ACCEPTED TO MEMORIAL IN SAVANNAH GA.

1537- WRITER SPOKE WITH BEN AT HEMS WHO STATES ETA OF 0100 FOR TRANSPORT.

1529-WRITER SPOKE WITH ERIC AT AIR EVAC TO POSSIBLY FLY PATIENT , DECLINED DUE TO WEATHER.

1532- WRITER ATTEMPTED TO CALL CONNECT EMS FOR TRANSPORT NO ANSWER,

1534- WRITER SPOKE WITH LEXI AT COAST TO COAST WHO STATES THEY HAVE NO ALS TRUCKS AVAILABLE.

1536- BEN WITH HEMS RETURNED CALL AND GAVE NEW ETA OF 1645 FOR TRANSPORT OF PATIENT TO MEMORIAL.). -- 15:43 09/22/26 EDT Cheryl Hulette, L.P.N.

15:52 09/22/26. (ATTEMPTED TO CALL REPORT TO 912-350-8113-WOULD NOT GO THRU-MULTIPLE ATTEMPTES. CALLED ONE STEP TRANSFER-RECEIVED 2 ALTERNATE PHONE NUMBERS). -- 16:08 09/22/26 EDT Lusha Fowler, R.N.

16:03 09/22/26. Zofran IVP 4 mg given via Site# 1. Allergies verified and confirmed 5 rights. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre-medication administration. IVP given by LPN. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 16:03 09/22/26 EDT Shalie Shiver, L.P.N.

16:03 09/22/26. MORPHINE IVP 2 mg given via Site# 1. Allergies verified and confirmed 5 rights. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre-medication administration. IVP given by LPN. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 16:03 09/22/26 EDT Shalie Shiver, L.P.N.

16:05 09/22/26. Bedside glucose 232 mg/dL; performed by nurse via finger. Result shown to the ED physician. -- 16:05 09/22/26 EDT Shalie Shiver, L.P.N.

18:00 09/22/26. MORPHINE IVP 2 mg given via Site# 1. Allergies verified and confirmed 5 rights. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre-medication administration. IVP given by nurse. Information reviewed with patient including reason for taking this medication, signs of allergic reaction and precautions. Verbalizes understanding. -- 18:01 09/22/26 EDT Lusha Fowler, R.N.

18:04 09/22/26. Bedside glucose 132 mg/dL; performed by nurse via finger. Result shown to the ED physician. -- 18:04 09/22/26 EDT Lusha Fowler, R.N.

19:03 09/22/26. Care transferred. (D.MONTGOMERY,LPN). -- 19:03 09/22/26 EDT Lusha Fowler, R.N.
19:39 09/22/26. (PT SITTING UP IN BED EATING APPLESAUCE AND CRACKERS NO S/S OF DISTRESS NOTED). -- 19:39 09/22/26 EDT Dale Montgomery, L.P.N.
19:39 09/22/26. BP: 120/68 MAP: 85. HR: 103. RR: 20. O2 saturation: 99% Temperature: 98.2 F. Pain level now 0/10. Glasgow Coma Scale: 15 - eyes open - spontaneous (4); best verbal response - oriented (5); best motor response - obeys commands (6). -- 19:39 09/22/26 EDT Dale Montgomery, L.P.N.
19:50 09/22/26. MORPHine IVP 2 mg given via Site# 1. Allergies verified and confirmed 5 rights. IV patency established. IV site checked: no pain, redness, or swelling. IV flushed thoroughly pre-medication administration. IVP given by nurse. Information reviewed with patient including reason for taking this medication, signs of allergic reaction, precautions and sedative warning. Verbalizes understanding. -- 19:55 09/22/26 EDT Dale Montgomery, L.P.N.
19:55 09/22/26. Site #1 in place upon transfer; patent, no pain, no signs of infection and no signs of infiltration. Good blood return present. Flushed with 10 mL saline; flushes easily. -- 19:56 09/22/26 EDT Dale Montgomery, L.P.N.
19:56 09/22/26. IV D5NS: Medication Continued. upon transfer at the rate of 100 mL/hr. 400 mL remaining in bag #1. IV patency established. IV site checked: no pain, redness, or swelling. -- 19:56 09/22/26 EDT Dale Montgomery, L.P.N.

DISPOSITION / DISCHARGE

16:05 09/22/26. Report was given to a nurse via a phone call. Report included information regarding patient's care and allergies and current vital signs. Report included treatment information regarding medications given in the ED. All questions were answered. Report was acknowledged (CLARE,RN). -- 16:07 09/22/26 EDT Lusha Fowler, R.N.
19:45 09/22/26. BP: 115/67 MAP: 83. HR: 107. RR: 20. O2 saturation: 99% Temperature: 98.2 F. Pain level now 0/10. Glasgow Coma Scale: 15 - eyes open - spontaneous (4); best verbal response - oriented (5); best motor response - obeys commands (6). -- 19:59 09/22/26 EDT Dale Montgomery, L.P.N.
19:57 09/22/26. BP: 118/76 MAP: 90. HR: 102. RR: 20. O2 saturation: 99% Temperature: 98.3 F. Pain level now 0/10. Glasgow Coma Scale: 15 - eyes open - spontaneous (4); best verbal response - oriented (5); best motor response - obeys commands (6). -- 19:59 09/22/26 EDT Dale Montgomery, L.P.N.
Departure time: 19:58 09/22/2026. Condition at departure: improved and stable. No learning barriers present. Transferred to memorial health savannah. Memorandum of Transfer, visit overview, Face Sheet, physician record and nurse record provided to EMS and transfer facility via paper. -- 19:58 09/22/26 EDT Dale Montgomery, L.P.N.

(Electronically signed by Dale Montgomery, L.P.N. 09/22/26 20:00:56 EDT)

Generated by CorroHealth

Superbill

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

09/22/2026 12:35:00 EDT

Patient: HARRIS, OLA
M

MRN#: A134531

Acct#: 561536

Sex: Female

DOB: 11/05/1969

Age: 56y

Category	Date/Time	Item Description	Facility Code	Professional Code	Quantity	Fee	Total
Nurse/IV/IM/Infusions	09/22/2026 12:44	IM/SQ (96372)	1490782		1	\$0.00	\$0.00
Nurse/IV/IM/Infusions	09/22/2026 12:46	IVP initial (96374)	1490784		1	\$0.00	\$0.00
Nurse/IV/IM/Infusions	09/22/2026 12:51	Hydration additional hour (96361)	1400085		7	\$0.00	\$0.00
Nurse/IV/IM/Infusions	09/22/2026 16:03	IVP additional push (96375)	1490793		2	\$0.00	\$0.00
Nurse/IV/IM/Infusions	09/22/2026 18:00	IVP same med (31 min apart) (96376)	1490795		2	\$0.00	\$0.00
Physician/Procedures	09/22/2026 19:54	Arterial Blood Gases (36600)	1436600		1	\$0.00	\$0.00
Nurse/E/M	09/22/2026 20:00	EMERGENCY DEPT VISIT HIGH SEVERITY&THREAT FUNCJ (99285-25)	9928525		1	\$0.00	\$0.00
						Grand Total	\$0.00

Providers

Michael Oates, M.D.

Chief Complaint

DECREASED MENTAL STATUS. PATIENT WITH HISTORY OF DIABETES AND CHRONIC RENAL FAILURE, PERITONEAL DIALYSIS, FOUND UNRESPONSIVE BY SON. EMS FOUND ACCU CHECK 24 GAVE IV AMP D50, ACCUCHECK 25 UPON ARRIVAL.

Principal Diagnosis

EVOLVING INFARCTION,
UNDERLYING AVM
HYPOGLYCEMIA.

Order Sheet

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

09/22/2026 12:35:00 EDT

Patient: HARRIS, OLA M MRN#: A134531 Acct#: 561536 Sex: Female DOB: 11/05/1969 Age: 56y

MEASUREMENTS: Wt: 90.7 kg

ALLERGIES: No known drug allergies

MEDICATION/IV/DRIP/FLUID ORDERS

Order Description	Priority	Entered	Acknowledged	Completed
IV D5NS at 100 mL/hr		13:17 09/22/2026 Michael Oates, M.D.		15:38 09/22/2026 Lusha Fowler, R.N.
Tylenol PO 650 mg		14:54 09/22/2026 Michael Oates, M.D.		15:12 09/22/2026 Lusha Fowler, R.N.
Dextrose 50%-Water IVP 25 g (NOW x1)		15:18 09/22/2026 Lusha Fowler, R.N. Verbal Order, Auth by: Michael Oates, M.D. Read back and verified		15:41 09/22/2026 Lusha Fowler, R.N.
Indication: HYPOGLYCEMIA				
Order Comments: 15:18 09/22/2026: (VO PRIOR TO FL ORDER) Lusha Fowler, R.N.				
Reason for ordering with alerts: NKDA --15:18 09/22/2026 Lusha Fowler, R.N.				
Glucagon IM 1 mg (NOW x1)		15:18 09/22/2026 Lusha Fowler, R.N. Verbal Order, Auth by: Michael Oates, M.D. Read back and verified		15:39 09/22/2026 Lusha Fowler, R.N.
Indication: patient with diabetes mellitus at risk of hypoglycemia				
Zofran IVP 4 mg		15:29 09/22/2026 Michael Oates, M.D.	16:03 09/22/2026 Shalie Shiver, L.P.N.	16:03 09/22/2026 Shalie Shiver, L.P.N.
Reason for ordering with alerts: Clinical consideration given --15:29 09/22/2026 Michael Oates, M.D.				
MORPHine IVP 2 mg		15:29 09/22/2026 Michael Oates, M.D.	16:03 09/22/2026 Shalie Shiver, L.P.N.	16:03 09/22/2026 Shalie Shiver, L.P.N.
Reason for ordering with alerts: Clinical consideration given --15:29 09/22/2026 Michael Oates, M.D.				
MORPHine IVP 2 mg (NOW x1)		17:56 09/22/2026 Michael Oates, M.D.		18:01 09/22/2026 Lusha Fowler, R.N.
Indication: pain				
Reason for ordering with alerts: Does not appear to be a true allergy --17:56 09/22/2026 Michael Oates, M.D.				
MORPHine IVP 2 mg (NOW)		19:54 09/22/2026 Dale Montgomery, L.P.N. Verbal Order, Auth by: Michael Oates, M.D. Read back and verified		19:55 09/22/2026 Dale Montgomery, L.P.N.
Indication: pain				
Reason for ordering with alerts: Benefits outweigh risks --19:54 09/22/2026 Dale Montgomery, L.P.N.				

LAB ORDERS

Order Description	Priority	Entered	Acknowledged	Collected	Completed
CBC Automated Stat	Stat	13:17 09/22/2026 Michael Oates, M.D.			12:45 09/22/2026 Lusha Fowler, R.N.
CMP Stat	Stat	13:17 09/22/2026 Michael Oates, M.D.			12:45 09/22/2026 Lusha Fowler, R.N.
BNP Stat	Stat	13:17 09/22/2026 Michael Oates, M.D.			12:45 09/22/2026 Lusha Fowler, R.N.
Urine Drug Screen Stat	Stat	13:17 09/22/2026 Michael Oates, M.D.	Cancelled: PT DOES NOT MAKE URINE.MD AWARE 15:48 EDT Lusha Fowler, R.N.		
Urinalysis w/ Reflex To Culture Stat	Stat	13:17 09/22/2026 Michael Oates, M.D.	Cancelled: PT DOES NOT MAKE URINE 15:48 EDT Lusha Fowler, R.N.		
High Sensitive Troponin Stat	Stat	13:18 09/22/2026 Michael Oates, M.D.			12:45 09/22/2026 Lusha Fowler, R.N.
PT with INR Stat	Stat	15:18 09/22/2026 Michael Oates, M.D.			12:45 09/22/2026 Lusha Fowler, R.N.

DIAGNOSTIC STUDY ORDERS

Order Description	Priority	Entered	Acknowledged	Completed
Chest 1V Port Stat	Stat	13:17 09/22/2026 Michael Oates, M.D.		13:45 09/22/2026 Lusha Fowler, R.N.
Reason for Study: Shortness of Breath				
Pregnancy Status: No				
Order Comments: 13:17 09/22/2026: Pregnancy Status: Not pregnant. Michael Oates, M.D.				
CT ABD/PEL wo IV Cont Stat	Stat	13:19 09/22/2026 Michael Oates, M.D.		13:45 09/22/2026 Lusha Fowler, R.N.
Reason for Study: Abdominal Pain				
Pregnancy Status: No				
CT Head wo IV Cont Stat	Stat	13:19 09/22/2026 Michael Oates, M.D.		13:45 09/22/2026 Lusha Fowler, R.N.
Reason for Study: Altered Sense of Awareness				
Pregnancy Status: No				

STAFF ORDERS

Order Description	Priority	Entered	Acknowledged	Collected	Completed
ABG Analysis Stat; (abg)	Stat	13:11 09/22/2026 Cindy Home Verbal Order, Auth by: Michael Oates, M.D. Read back and verified	13:11 09/22/2026 Cindy Home		15:53 09/22/2026 Lusha Fowler, R.N.
Order Comments: 13:11 09/22/2026: (100%o2) Cindy Home					
EKG Stat	Stat	13:11 09/22/2026 Cindy Home Verbal Order, Auth by: Michael Oates, M.D. Read back and verified	13:11 09/22/2026 Cindy Home		13:15 09/22/2026 Lusha Fowler, R.N.
POC Glucose		13:37 09/22/2026 Michael Oates, M.D.			12:42 09/22/2026 Lusha Fowler, R.N.

[Electronically signed by Michael Oates, M.D. (09/22/2026 13:20 EDT)]
[Electronically signed by Michael Oates, M.D. (09/22/2026 13:38 EDT)]
[Electronically signed by Michael Oates, M.D. (09/22/2026 14:54 EDT)]
[Electronically signed by Michael Oates, M.D. (09/22/2026 14:54 EDT)]
[Electronically signed by Michael Oates, M.D. (09/22/2026 15:18 EDT)]
[Electronically signed by Michael Oates, M.D. (09/22/2026 15:29 EDT)]
[Electronically signed by Michael Oates, M.D. (09/22/2026 17:56 EDT)]
[Electronically signed by Michael Oates, M.D. (09/22/2026 17:56 EDT)]
[Electronically signed by Michael Oates, M.D. (09/22/2026 19:54 EDT)]

Visit Overview

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

09/22/2026 12:35:00 EDT

Patient: HARRIS, OLA
M

MRN#: A134531

Acct#: 561536

Sex: Female

DOB: 11/05/1969

Age: 56y

09/22/2026 08:01 PM EDT

ED Arrival: 12:35 09/22/2026 EDT	Pregnancy Status: not pregnant	Recent Travel: no
Language: ENGLISH	Adv Directive:	Isolation Status:
Ethnicity: Not Hispanic or Latino	Fall Risk:	Infectious Disease Exposure: no
Measurements: 200.0 lb / 90.7 kg	Self-Harm Status: risk	Sepsis Screen:
Chief Complaint: UNRESPONSIVE and (IV x2. AMBUING PT.)		

ALLERGIES

- No Known Drug Allergies

HOME MEDICATIONS

- albuterol sulfate HFA 90 mcg/actuation aerosol inhaler: as needed.
- Breztri Aerosphere inhalation
- Brilinta 90 mg tablet: 90 mg twice a day .
- calcitriol 0.5 mcg capsule: 0.5 mcg once a day .
- citalopram 20 mg tablet: 20 mg once a day .
- Fish Oil 1,000 mg (120 mg-180 mg) capsule: 1 cap once a day .
- HumaLOG KwikPen Insulin subcutaneous: (SLIDING SCALE)
- Lantus Solostar U-100 Insulin 100 unit/mL (3 mL) subcutaneous pen: 75 unit twice a day for type 2 diabetes mellitus.
- Lipitor 40 mg tablet: 40 mg at bedtime .
- megestrol 40 mg tablet: 40 mg once a day .
- metoprolol succinate ER 50 mg tablet,extended release 24 hr: 50 mg at bedtime .
- NIFEdipine ER 30 mg tablet,extended release: 30 mg once a day .
- Norvasc 5 mg tablet: 5 mg once a day .
- omeprazole 20 mg tablet,delayed release: 40 mg once a day .
- potassium chloride ER 20 mEq tablet,extended release: 20 mEq once a day .
- pregabalin 150 mg capsule: 150 mg twice a day .
- pregabalin 25 mg capsule: 25 mg once a day .
- QUEtiapine 300 mg tablet: 150 mg at bedtime .
- Singulair 10 mg tablet: 10 mg at bedtime .
- Xphozah 30 mg tablet: 30 mg twice a day .

PAST MEDICAL HISTORY / PROBLEMS

- COPD - Chronic Obstructive Pulmonary Disease
- Diabetes Mellitus
- Gastroparesis
- HIGH CHOLESTEROL
- Hypertension
- LNMP: The patient is post-menopausal
- Peritoneal dialysis (procedure)
- Renal Failure

PAST SURGICAL HISTORY

- Unknown

SOCIAL HISTORY

- Nutritional assessment: No deficits
- Functional assessment: No impairments
- Learning needs: No barriers
- Smoking status: Unknown

ED COURSE**MEDICATIONS GIVEN IN EMERGENCY****DEPARTMENT**

12:44 09/22/26 Glucagon IM 1 mg
 12:46 09/22/26 Dextrose 50%-Water IVP 25 g
 12:51 09/22/26 IV D5NS 100 mL/hr
 15:06 09/22/26 Tylenol PO 650 mg
 16:03 09/22/26 Zofran IVP 4 mg
 16:03 09/22/26 MORPHine IVP 2 mg
 18:00 09/22/26 MORPHine IVP 2 mg
 19:50 09/22/26 MORPHine IVP 2 mg

IV SITE INFORMATION

12:45 09/22/26 Site #1 left EJ, 18g. Saline lock.

INTAKE**OUTPUT****REASSESSMENT (most recent)**

- 12:35 09/22/26. To room via stretcher.
 GENERAL / NEURO / PSYCH: (PT UNRESPONSIVE. EMS BAGGING PT).
 RESPIRATORY: Moderate respiratory distress. (RESP M< 10 BPM PER EMS). CVS: Cardiac rhythm: sinus tachycardia; (SINUS TACH WITH FREQUENT PVC's). Capillary refill less than 2 seconds. GI / GU: Abdomen soft. SKIN: Skin is warm and dry. Poor skin turgor.

VITAL SIGNS

	First Vitals		Last Vitals	
Temp	12:45 09/22/26	93.2 F	19:57 09/22/26	98.3 F
BP	12:45 09/22/26	140/80	19:57 09/22/26	118/76
HR	12:45 09/22/26	130	19:57 09/22/26	102
RR	12:45 09/22/26	12	19:57 09/22/26	20
O2 Sat	12:45 09/22/26		19:57 09/22/26	99%
Pain	12:45 09/22/26	0	19:57 09/22/26	0
ETCO2	12:45 09/22/26		19:57 09/22/26	
GCS	12:45 09/22/26	10	19:57 09/22/26	15
RTS	12:45 09/22/26		19:57 09/22/26	

PROCEDURES

NURSING INTERVENTIONS

LABS / STUDIES

LABS / STUDIES ORDERED

- BNP
- CBC Automated
- Chest 1V Port
- CMP
- CT ABD/PEL wo IV Cont
- CT Head wo IV Cont
- High Sensitive Troponin
- PT with INR

LABS / STUDIES PENDING IMPORT

- CHEST 1 VIEW

CLINICAL IMPRESSION

Facility Coding Summary

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

09/22/2026 12:35:00 EDT

Patient: HARRIS, OLA	MRN#:	Acct#:	Sex:	DOB:	Age:
M	A134531	561536	Female	11/05/1969	56y

Providers: Michael Oates, M.D.

DIAGNOSTIC WORKUP

Chief Complaint

DECREASED MENTAL STATUS. -- Michael Oates, M.D.

PATIENT WITH HISTORY OF DIABETES AND CHRONIC RENAL FAILURE, PERITONEAL DIALYSIS, FOUND UNRESPONSIVE BY SON. EMS FOUND ACCU CHECK 24 GAVE IV AMP D50, ACCUCHECK 25 UPON ARRIVAL.

-- Michael Oates, M.D.

Principal Diagnosis

ICD-10 Codes

PROCEDURES

Procedures from Providers:

Procedures from Nurses/Facility:

- Arterial Blood Gases (CPT: 36600)
- Arterial Blood Gases (CPT: 36600-76)
- Bedside Glucose (CPT: 82962)
- Bedside Glucose (CPT: 82962-91 X3)
- IV Push Dextrose 50%-Water IVP (CPT: 96374)
- IV Push Zofran IVP (CPT: 96375)
- IV Push MORPHine IVP (CPT: 96375)
- IV Push MORPHine IVP (CPT: 96376)
- IV Push MORPHine IVP (CPT: 96376)
- Injection Glucagon IM (CPT: 96372)
- IV Hydration (CPT: 96361 X7)

SUPPLIES

E&M LEVEL

99285-25

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Milestone Report

Bleckley Memorial Hospital

145 E Peacock St. Cochran, GA 31014 4789346211

09/22/2026 12:35:00 EDT

Patient: HARRIS, OLA	MRN#:	Acct#:	Sex:	DOB:	Age:
M	A134531	561536	Female	11/05/1969	56y

Milestones	Time	Date	Elapsed Time
Arrival	12:35	09/22/2026	00:00
Triage	12:35	09/22/2026	00:00
To Treatment Room	12:35	09/22/2026	00:00
Seen by Provider			
Disposition Decision	15:32	09/22/2026	02:57
Departure	19:58	09/22/2026	07:23